



Government of Western Australia
Child and Adolescent Health Service



Child and Adolescent Mental Health Service

CAMHS Crisis Connect Model of Care



Compassion

Excellence

Collaboration

Accountability

Equity

Respect

Document Control

Method of development

This model of care has been developed in consultation with CAMHS and PCH operational representatives, the CAMHS Leadership Group and other stakeholders where indicated.

Distribution for consultation

Version	Authors / editors	Date distributed	Recipients
Draft 0.1	Beverley Robison, Dr Alex Thompson	22 January 2021	Imogen Davies
Draft 0.2	Beverley Robison, Imogen Davies	2 February 2021	Katie MacKenzie, Dr Pradeep Rao, Dr Simon Davies, Sue Baker, Dr Jacques Esterhuizen, Dr Alex Thompson, Marina Whittaker, Dr Michael Baker, Craig Hasler, Gabrielle Anstey, Dr Vineet Padmanabhan, Roger Golling, Kay Hlaing, Rachel Newton, Kara Edwards
Draft 0.3	Beverley Robison, Dr Alex Thompson	3 March 2021	Dr Jacques Esterhuizen, Sue Baker
Draft 0.4	Beverley Robison, Sue Baker	24 March 2021	CAMHS Service Managers and CAMHS Heads of Service (CAMHS Cross-Directorate)
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Draft 0.7	Pradeep Rao, Katie McKenzie	19 April 2021	N/A – Changes requested by PR have been made
Draft 0.8	Dr Alex Thompson	September 2021	Dr Jacques Esterhuizen, Karen Curtis
Draft 0.9	Marina Whittaker, Sarah MacDonald, Kylie McDonald	April 2023	Dr Emma Gill, Karen Curtis, Dr Vineet Padmanabhan

Endorsed by CAMHS Executive Clinical Governance Committee 13/9/2023

***Note: CAMHS Director Clinical Services approved a 3-month extension of the review date to 06/06/2026.**

1. Context

This document outlines the proposed Model of Care (MoC) for Child and Adolescent Mental Health Service Crisis Connect (CAMHS CC).

This MoC has been developed in response to the *Chief Psychiatrist's Targeted Review into the Treatment of Ms Kate Savage by Child and Adolescent Mental Health Services* [the Review].¹ Recommendation 5 in the Review calls for the establishment of a 24-hour, 7 day multi-disciplinary, emergency mental health service at PCH.

CAMHS CC provides a single point of contact for children in community settings requiring mental health support, brief intervention, assessment, including children presenting to metropolitan Emergency Departments (EDs). The service also provides mental health assessments for children presenting to Perth Children's Hospital (PCH) ED with immediate or acute suspected mental health conditions and coordinates referrals and admissions to CAMHS Ward 5A. The CAMHS Crisis Connect Intervention provides short term intervention to suitable young people who have presented to a metropolitan ED following a mental health crisis. (See Appendix A for CAMHS CC Intervention Pathway).

An evaluation of service performance will occur with reports generated on a six-monthly basis. Reports will be tabled at CAMHS Executive Clinical Governance Committee.

2. Service aim and alignment

Service aim

CAMHS CC aims to improve the experience of young people, caregivers and families experiencing a mental health crisis by providing care closer to home, and providing the right care, at the right place at the right time.

CAMHS CC will improve patient flow by;

- promoting the diversion of patients requiring an emergency mental health assessment (crisis presentations) from attending a metropolitan ED by offering an alternative service via tele-mental health;
- improving efficiencies when children arrive at ED for a mental health assessment thereby reducing the percentage of re-presentations to PCH and other metropolitan EDs; and
- referring children and families to appropriate services.

Strategic alignment

CAMHS CC aligns strategically with the following publications:

- *National Framework for Recovery-Oriented Mental Health Services* (2013)
- *The Fifth National Mental Health and Suicide Prevention Plan* (2017)
- *Mental Health Act 2014*
- *Better Choices, Better Lives: Western Australian Mental Health, Alcohol and Other Drugs Services Plan 2015-2025* (2015)
- *WA State Priorities Mental Health, Alcohol and Other Drugs 2020-2024* (2020)
- *WA Health Sustainable Health Review* (2019)

Guiding principles

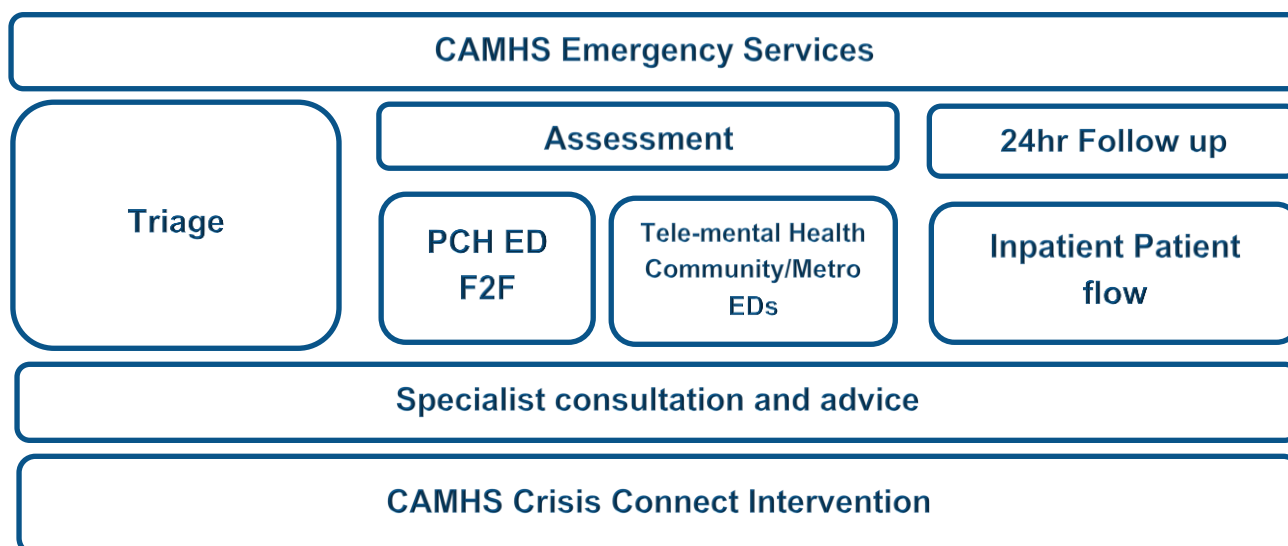
CAMHS CC is guided by the *Mental Health Act 2014* Charter of Mental Health Care Principles. In addition, the service ensures consumer and carer involvement; least restrictive practice; and recovery focussed care.

3. Service delivery

Service components

Figure 1 shows the service components provided by CAMHS CC.

Figure 1. Components of CAMHS CC service delivery.



1. Specialist urgent tele-mental health support and triage

Specialist tele-mental health urgent support and crisis intervention for young people, parents and carers in crisis, and professionals in the community.

2. Face to face (F2F) mental health and risk assessment at PCH ED

Mental health assessments for young people who present to the PCH ED and require an urgent mental health assessment.

3. Tele-mental health assessment

Where appropriate, tele-mental health assessment as an alternative to presentation at PCH or another metropolitan ED.

4. 24 hour Follow up

Telephone follow up contact of families within 24 hours of receiving a mental health assessment (in PCH ED or via tele-mental health); and/or

5. Child and adolescent state-wide mental health patient flow

The co-ordination of referrals and admissions to Ward 5A.

6. Specialist consultation, advice and guidance

Specialist emergency mental health consultation, advice, guidance and liaison with other professionals at metropolitan EDs and in the community, for example GPs, school psychologists, school nurses.

7. CAMHS Crisis Connect Intervention

A post ED follow up clinic which provides short term intervention to suitable young people (0-15 years of age), that have presented to a metropolitan ED following a mental health crisis. The purpose is to provide time-limited psychological therapy (4-6 sessions over 2 weeks) aimed at addressing the presenting issue identified at referral and the immediate needs of the young person.

Inclusion criteria

- Young people under the age of 18 years old requiring urgent mental health support or mental health assessment while in a community setting such as a school or non-government organisation.
- Families, carers and professionals in the community seeking mental health support and guidance about a young person under the age of 18.
- Young people under the age of 16 years old presenting to PCH ED requiring an urgent mental health assessment.
- Young people under the age of 16 years old presenting at another metropolitan ED requiring an urgent mental health assessment. (Young people presenting to another metropolitan ED will be seen first by the mental health clinician in the local ED. CAMHS CC can provide urgent tele-mental health support and assessment as indicated).
- Young people under the age of 16 who have had a mental health assessment by CAMHS CC at PCH or another metropolitan ED may be referred to CAMHS Crisis Connect Intervention.

In rural, regional and remote areas of Western Australia, the WA Country Health Service (WACHS) Mental Health Emergency Telehealth Service (MH ETS) provides WA country doctors and nurses with access to specialist clinicians via video conferencing technology 24 hours a day, 7 days a week.

Hours of operation

CAMHS CC operates 24 hours a day, 7 days a week.

Patient pathways and care continuity

Referrals are accepted from a range of sources, including schools, GPs, Department of Communities, private clinicians and non-government organisations. Referrals to CAMHS Crisis Connect Intervention will be via CAMHS CC following an assessment at either PCH ED or via tele-mental health.

PCH ED mental health assessments are completed by either CAMHS CC CNS or Psychiatry Registrar. Where possible, patients who initially contact CAMHS CC and subsequently present at ED, will receive their face-to-face mental health assessment by the clinician they spoke with on the phone. Mental health patients who present to PCH ED receive a medical assessment from an ED physician, either before or after the mental health assessment.

CAMHS CC staff will complete all Risk Assessments and Management Plans (RAMPs) in ED for patients being admitted. Complex ED presentations may receive input from the PCH ED Social Worker.

CAMHS CC works closely with other entities to ensure care continuity and support patients receiving emergency care in the least restrictive environment. Notable interfaces with other services include:

- Community CAMHS
- WA Country Health Service (WACHS)
- PCH Ward 5A (Mental Health Inpatient Unit) and Ward 4A (including the Eating Disorders pod)
- WA Police

- Department of Child and Family Services
- Department of Education
- Youth inpatient mental health wards (EMyU or FSH)
- Not-for-profit mental health, alcohol and/or other drug services.

4. Workforce

CAMHS CC is staffed by a multidisciplinary team of clinical staff including medical, nursing and allied health and Aboriginal Mental Health Workers who will provide culturally appropriate care where appropriate. The resourcing requirements demonstrate that ED face to face assessments are a core function of the service, and that adequate staffing (with appropriate skills) are essential to ensuring timely ED assessment.

Psychiatric registrar availability outside of normal working hours

Outside of business hours (ie.1700 – 0900, during weekends and public holidays) the Duty Psychiatric Registrar may be contacted when demand in PCH ED outweighs CAMHS CC capacity to conduct mental health assessments in a clinically appropriate timeframe.

This decision must be made in collaboration with the On-call Consultant Psychiatrist. Ward 5A responsibilities are the registrar's priority; they may not always be available to attend ED. NB. Until a dedicated CAMHS CC Psychiatric Registrar is recruited, the Duty Psychiatry Registrar may be contacted during working hours with CAMHS CC Consultant Psychiatrist approval.

5. Governance, Safety and Quality

CAMHS Crisis Connect will be aligned with the PCH CAMHS directorate. CAMHS Crisis Connect will be led by a CAMHS Head of Service and CAMHS Clinical Nurse Manager/Consultant who will report to the CAMHS Director Clinical Services via the Director PCH CAMHS and Head of Department PCH CAMHS. CAMHS Crisis Connect Business, Governance and Reflective Space Group provides a forum for clinicians and leadership to review service performance, safety and quality whilst also providing an opportunity to reflect on clinical practice. CAMHS Crisis Connect will also utilise existing CAMHS and CAHS Committee structures to report on performance, safety and quality.

Service provision and quality development is guided by the National Safety and Quality Health Service Standards, the National Standards for Mental Health Services and the Chief Psychiatrist's Standards for Clinical Care. CAMHS Crisis Connect are also guided by the Royal Australian and New Zealand College of Psychiatrists' Professional Practice Standards and Guides for Telepsychiatry.

CAMHS Crisis Connect clinicians utilise the State-wide Standardised Clinical Documentation suite and record patient information in the Psychiatric Online Information System (PSOLIS). All staff undertake approved safety and quality activities on a regular basis.

6. Education, Research and Evaluation

CAMHS Crisis Connect will undertake ongoing service evaluation and be supported to undertake research in the area of acute mental health care in children and adolescents.

Key Performance Indicators

Two Key Performance Indicators (KPI's) will be reported:

- 90% of children accessing Perth Children's Hospital (PCH) for a mental health assessment are transferred/discharged within 4 hours of arrival²
- 100% of children accepting follow-up contact from CAMHS Emergency Service (Crisis Connect) will be contacted within 24 hours of discharge/assessment³

Evaluation Measures

A number of measures are available for effective evaluation and ongoing monitoring of CAMHS Crisis Connect service performance. Dashboards development or refinement enables adjustable reporting depending on purpose and audience. This includes:

- Absolute number, mean and median reporting
- Time periods (data is captured daily and some measures are available pre-CAMHS ETS/Crisis Connect allowing for performance comparisons)

Incoming calls

Numbers of calls

- Duration of calls
- Client demographics (including gender, age, type (professional/client), and location)
- Reason for call (Information only/Seeking assistance)
- Abandoned calls

This KPI is consistent with the West Australian Emergency Access Target (WEAT).

3 This measure will consider that not all families offered follow up will accept follow up; and some families may request follow-up at a time/date after 24 hours have expired.

- Diverted calls
- Queue times

Emergency Departments

The number of children presenting to PCH ED and Metropolitan ED mental health (including by time of day)

- The number children presenting to PCH ED and Metropolitan ED following CAMHS ETS contact
- Length of Stay in PCH ED
- PCH West Australian Emergency Access Target (WEAT)

Crisis Connect Assessments

- Number
- Duration
- Mode (face to face/tele-health)
- Assessment outcome (referred to Ward 5A, referred to Community CAMHS teams or other CAMHS teams, referred to other government services)

Follow ups (post inpatient discharge and post assessment)

- Proportion of children who accept follow up
- Length of follow up calls
- Timeliness of follow up
- Follow up outcomes

ED Diversion

- Number of children directed to PCH ED by Community CAMHS
- Number of direct admissions to Ward 5A not requiring ED presentation (by referrer type: Community CAMHS, Private Psychologist, other)
- Percentage of presentations to PCH ED following access to CAMHS Crisis Connect helpline

Patient experience and outcomes

- Patient Reported Experience Measure (PREM) and
- Patient Reported Outcome Measure (PROM)

Appendix A: CAMHS CC Intervention Pathway

